

FAX TRANSMISSION COVER SHEET

Ridgeline Orthopedic & Spine Associates

4420 Meridian Parkway, Suite 210 | Fax: (614) 555-0198 | Phone: (614) 555-0142

TO:	Concord Utilization Management
DATE:	09/24/2026
FAX:	1-800-555-0177
PAGES:	14 (including cover)
FROM:	Dr. Amanda Reyes, MD
RE:	Prior Authorization Request
PATIENT:	Doe, John
MEMBER ID:	MBR-0042

URGENT — Prior authorization requested for scheduled inpatient admission.

Attached please find the surgical consultation, history and physical, cardiology clearance, pre-operative orders, and laboratory results supporting the above-referenced request. Please contact our office with any questions regarding this submission.

CONFIDENTIALITY NOTICE: This facsimile transmission contains confidential health information protected under HIPAA. If you are not the intended recipient, please notify the sender immediately and destroy this transmission.

Prior Authorization Request Form

Concord Utilization Management Program • Page 2 of 14

Member Name:	John Doe
Member ID:	MBR-0042
Date of Birth:	03/14/1955
Plan:	Concord Gold Plus HMO
Requesting Provider:	Amanda Reyes, MD
NPI:	1780234561
Facility:	Ridgeline Surgical Center
Facility NPI:	1922004471
Requested Service:	Elective inpatient admission
Procedure:	Open lumbar spinal fusion, L4-L5
CPT:	22612
Requested Admission Date:	10/02/2026
Requested LOS:	See attached notes
Diagnosis:	Lumbar spondylolisthesis with radiculopathy
ICD-10:	M43.16

Clinical Justification Summary

Patient is a 71-year-old male with progressive lumbar radiculopathy refractory to conservative management (physical therapy x 12 weeks, epidural steroid injection x2). MRI demonstrates severe L4-L5 spondylolisthesis with foraminal stenosis. Surgical intervention recommended given failure of conservative treatment and progressive neurologic symptoms. See attached surgical consultation, H&P, and cardiology clearance for full clinical detail.

Member Eligibility & Benefits Verification

Generated by Ridgeline Surgical Center — Patient Access • Page 3 of 14

Member Name:	John Doe
Member ID:	MBR-0042
Group Number:	GRP-88214
Effective Date:	01/01/2026
Plan Type:	Concord Gold Plus HMO (Medicare Advantage)
Prior Auth Required:	Yes — inpatient surgical admission
Verification Date:	09/20/2026
Verified By:	K. Alvarez, Patient Access Rep

Note: Eligibility and benefit information is verified as of the date shown above and is subject to change. Verification does not guarantee payment.

Surgical Consultation Note

Ridgeline Orthopedic & Spine Associates • Page 4 of 14

Patient: John Doe | DOB: 03/14/1955 | Date of Visit: 09/18/2026

Chief Complaint

Progressive low back pain with right leg radiculopathy, 8 months duration.

History of Present Illness

Mr. Doe is a 71-year-old male referred for evaluation of chronic low back pain with radiation into the right lower extremity. Symptoms have progressively worsened over the past 8 months despite a course of physical therapy and two epidural steroid injections. MRI lumbar spine dated 08/15/2026 demonstrates Grade 1 spondylolisthesis at L4-L5 with associated foraminal stenosis and nerve root impingement. Patient reports increasing difficulty with ambulation and activities of daily living.

Past Medical History

Congestive heart failure (reduced ejection fraction, last EF 40% per cardiology), Type 2 diabetes mellitus, hypertension, hyperlipidemia.

Assessment

Lumbar spondylolisthesis with radiculopathy, refractory to conservative management. Surgical candidate for decompression and fusion.

Plan

Recommend open lumbar spinal fusion, L4-L5, with posterior instrumentation. Given patient's cardiac history (reduced EF) and diabetes, cardiology clearance obtained prior to scheduling — see attached. Surgeon expects a multi-day stay for pain control and mobilization. Patient counseled on risks, benefits, and alternatives; agrees to proceed.

Electronically signed: Amanda Reyes, MD | 09/18/2026 4:42 PM

Surgical Consultation Note (continued)

Ridgeline Orthopedic & Spine Associates • Page 5 of 14

Review of Systems

Constitutional: Denies fever, chills, unintentional weight loss. Cardiovascular: Denies chest pain, orthopnea. Reports mild exertional dyspnea, stable. Endocrine: Diabetes controlled on oral agents, last A1c 7.1%. Musculoskeletal: As above.

Physical Examination

Vitals: BP 138/82, HR 76, RR 16, SpO2 97% RA. Lumbar spine with decreased range of motion, positive straight leg raise on right. Motor strength 4/5 right EHL. Sensation diminished right L5 dermatome. Reflexes 1+ bilateral patellar, absent right Achilles.

Imaging Reviewed

MRI Lumbar Spine (08/15/2026): Grade 1 anterolisthesis L4 on L5, severe bilateral foraminal stenosis, moderate central canal stenosis.

History & Physical

Patient: John Doe | MRN: 774213 | Date: 09/22/2026

Reason for Admission

Pre-operative history and physical for scheduled open lumbar spinal fusion, L4-L5.

Past Medical History

1. Congestive heart failure, HFrEF, EF 40% (echo 07/2026)
2. Type 2 diabetes mellitus, on metformin and glipizide
3. Hypertension, controlled on lisinopril
4. Hyperlipidemia, on atorvastatin
5. GERD

Past Surgical History

Cholecystectomy (2011), right total knee arthroplasty (2019).

Medications

Metformin 1000mg BID, Glipizide 5mg daily, Lisinopril 20mg daily, Atorvastatin 40mg daily, Furosemide 20mg daily, Aspirin 81mg daily.

Allergies

No known drug allergies.

History & Physical

Social History

Former smoker, quit 2015 (30 pack-year history). Denies alcohol or illicit substance use. Lives with spouse, independent with ADLs at baseline.

Family History

Father: coronary artery disease. Mother: type 2 diabetes.

Physical Examination

General: Well-developed, well-nourished male in no acute distress.

Cardiovascular: Regular rate and rhythm, no murmurs. Trace bilateral lower extremity edema.

Pulmonary: Clear to auscultation bilaterally, no wheezes or crackles.

Abdomen: Soft, non-tender, non-distended.

Extremities: No cyanosis or clubbing. Trace edema as noted above.

Neurologic: As detailed in surgical consultation note.

Functional Status

Patient ambulates with a single-point cane due to pain, otherwise independent. Lives in a two-story home; will require assistance with stairs post-operatively.

History & Physical

Pre-Admission Testing Summary

See attached laboratory results (page 13) and cardiology clearance (pages 9-10) for complete pre-operative risk stratification.

Anesthesia Risk Assessment

ASA Physical Status Classification: III (severe systemic disease — CHF, diabetes). Anesthesia pre-op evaluation completed 09/23/2026, cleared for surgery with standard cardiac monitoring intra-operatively.

Assessment / Plan

71-year-old male with HFrEF (EF 40%) and type 2 diabetes, cleared for open lumbar spinal fusion following cardiology evaluation. Anticipate inpatient admission for post-operative monitoring given cardiac history; exact anticipated length of stay to be determined by surgical team based on intra-operative course and post-operative mobilization.

Cardiology Pre-Operative Clearance

Heartland Cardiology Associates • Page 9 of 14

Patient: John Doe | DOB: 03/14/1955 | Date: 09/16/2026

Reason for Consultation

Pre-operative cardiac risk assessment prior to elective lumbar spinal fusion.

History

Patient with known history of heart failure with reduced ejection fraction, last assessed at 40% on echocardiogram dated 07/09/2026. NYHA Class II symptoms, stable on current guideline-directed medical therapy. No recent decompensation, hospitalization, or change in symptoms.

Diagnostic Studies

Echocardiogram (07/09/2026): LVEF 40%, mild-moderate mitral regurgitation, no significant valvular stenosis.

EKG (09/16/2026): Normal sinus rhythm, no acute ST-T changes.

Stress test: Not indicated given recent echo and stable symptoms.

Assessment

Revised Cardiac Risk Index: 2 points (heart failure, diabetes). Estimated peri-operative cardiac risk: intermediate. Patient's heart failure is compensated and stable.

Cardiology Pre-Operative Clearance (continued)

Heartland Cardiology Associates • Page 10 of 14

Recommendations

1. Patient is cleared for surgery from a cardiac standpoint.
2. Continue home cardiac medications through peri-operative period per anesthesia protocol.
3. Recommend telemetry monitoring for first 24-48 hours post-operatively given HFrEF.
4. Maintain euvolemia — monitor daily weights and strict intake/output.
5. Given cardiac history, inpatient monitoring is recommended; anticipate this will support a multi-day admission for adequate post-operative cardiac and pain management, though exact duration will depend on post-operative course.

Clearance Status: CLEARED FOR SURGERY

Electronically signed: Dr. Priya Nkemelu, MD, FACC | 09/16/2026

Pre-Operative Orders

Ridgeline Surgical Center • Page 11 of 14

Procedure:	Open lumbar spinal fusion, L4-L5
Scheduled Date:	10/02/2026
Admission Type:	Inpatient
Attending Surgeon:	Amanda Reyes, MD

Pre-Operative Instructions

1. NPO after midnight prior to surgery.
2. Continue metformin, lisinopril, atorvastatin morning of surgery with sip of water.
3. Hold glipizide morning of surgery.
4. Type and screen, CBC, BMP, coagulation panel day of surgery.
5. Cardiology clearance on file — see attached.
6. Anesthesia pre-op evaluation on file — see H&P.
7. Sequential compression devices intra-operatively for DVT prophylaxis.

Pre-Operative Orders (continued)

Ridgeline Surgical Center • Page 12 of 14

Post-Operative Plan (Anticipated)

Patient anticipated to require inpatient admission post-operatively for pain control, mobilization, and cardiac monitoring given history of heart failure. Physical therapy consult day of surgery. Telemetry monitoring per cardiology recommendation. Diabetic management per sliding scale insulin protocol while NPO/post-operative. Discharge planning to begin day 1 post-operatively; anticipated discharge disposition to be determined based on mobility and pain control status.

DVT Prophylaxis

Sequential compression devices + subcutaneous heparin per protocol, starting post-operative day 0.

Laboratory Results

Ridgeline Surgical Center — Pre-Admission Testing • Page 13 of 14

Collected: 09/23/2026

Test	Result	Reference Range	Flag
WBC	6.8 K/uL	4.5-11.0	
Hemoglobin	13.2 g/dL	13.5-17.5	L
Platelets	245 K/uL	150-400	
Sodium	138 mEq/L	136-145	
Potassium	4.4 mEq/L	3.5-5.1	
Creatinine	1.1 mg/dL	0.7-1.3	
Glucose	142 mg/dL	70-99	H
Hemoglobin A1c	7.1 %	<5.7	H
INR	1.0	0.8-1.1	
BNP	210 pg/mL	<100	H

Interpretation

Mild anemia, consistent with chronic disease. Hyperglycemia and elevated A1c consistent with known type 2 diabetes, adequate for surgical clearance per anesthesia. BNP mildly elevated, consistent with known compensated heart failure — no acute change from prior baseline.

Provider Attestation & Certification of Medical Necessity

Ridgeline Orthopedic & Spine Associates • Page 14 of 14

I certify that the information provided in this prior authorization request, and the attached clinical documentation, is accurate and reflects the current medical condition of the patient named herein. I attest that the requested inpatient admission for open lumbar spinal fusion (L4-L5) is medically necessary based on the patient's clinical presentation, failed conservative management, and comorbid conditions requiring inpatient-level post-operative monitoring.

Provider Signature: Amanda Reyes, MD

Date: 09/24/2026

NPI: 1780234561

— End of transmission (14 pages) —